**Clinical Study Summary Report (CSSR) Document Status:** Draft | **Version:** 0.1 | **Date:** 04-Apr-2026

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## 1. Administrative & Study Identification

**Full Study Title:** A Phase III Renal Outcomes and Cardiovascular Mortality Study to Investigate the Efficacy and Safety of Baxdrostat in Combination with Dapagliflozin in Participants with Chronic Kidney Disease and High Blood Pressure **Short Title/ Acronym:** BaxDuo-Pacific (PACIFIC Study) **Protocol Number:** D6972C00002 **NCT Number:** NCT06742723 **Sponsor Name:** AstraZeneca **Investigational Product:** Baxdrostat in combination with dapagliflozin (Drug Name: Dapagliflozin | ATC Code: A10BK01 | Trade Names: Qternmet, Qtern, Xigduo, Farxiga) / Placebo in combination with dapagliflozin **Phase of Development:** Phase III **Medical Monitor:** AstraZeneca Clinical Operations Lead

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## 2. Study Synopsis & Rationale

**2.1 Study Objective Primary Objective:** To investigate the efficacy, safety, and tolerability of baxdrostat in combination with dapagliflozin, compared with placebo and dapagliflozin, in reducing the risk of the composite endpoint of $\ge 50\%$ sustained decline in eGFR, kidney failure, or cardiovascular (CV) death in participants with Chronic Kidney Disease (CKD) and hypertension (HTN). **Secondary Objectives:** To determine superiority in reducing UACR after 16 weeks, reducing SBP after 16 weeks, and reducing the risk of Major Adverse Cardiac Events (MACE).

**2.2 Background & Rationale** To address the high risk of kidney failure and serious cardiovascular events in adults living with CKD and high blood pressure despite standard of care therapies. Baxdrostat is a highly selective aldosterone synthase inhibitor (ASI) designed to lower blood pressure by inhibiting the production of aldosterone. The PACIFIC study investigates whether combining baxdrostat with dapagliflozin (an SGLT2 inhibitor) can further reduce the risks of kidney failure and heart problems compared to taking dapagliflozin alone.

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## 3. Study Design & Methodology

**3.1 Design Framework Study Type:** Interventional **Control Type:** Placebo-controlled **Blinding:** Double-blind **Randomization:** Randomized (Parallel Assignment) **Trial Status:** RECRUITING

**3.2 Planned Enrollment & Duration Target $N$:** 5,000 participants **Number of Sites:** International, Multicenter **Estimated Study Start:** 03-Mar-2025 **Estimated Primary Completion:** 18-Dec-2029

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## 4. Subject Selection (Eligibility Criteria)

**4.1 Inclusion Criteria** 1. Participants of any sex and gender must be $\ge 18$ years of age at the time of signing informed consent. 2. Participants with (a) eGFR 30-59 mL/min/1.73 m2 AND UACR $\ge 30$ mg/g and $< 500$ mg/g, or UACR $\ge 500$ mg/g and $\le 5000$ mg/g, or UPCR $\ge 700$ mg/g and $\le 7000$ mg/g; OR (b) eGFR 60-75 mL/min/1.73 m2 AND UACR $\ge 500$ mg/g and $\le 5000$ mg/g, or UPCR $\ge 700$ mg/g and $\le 7000$ mg/g. 3. History of HTN and a systolic blood pressure (SBP) $\ge 130$ mmHg (the most recent value within 4 weeks prior to screening or at screening) and $\ge 120$ mmHg at the Randomisation Visit. 4. Stable and maximum tolerated dose of an ACE inhibitor or an ARB (not both) for at least 4 weeks prior to the Screening Visit. 5. Serum or plasma potassium $\ge 3.0$ and $\le 4.8$ mmol/L if eGFR $\ge 45$ mL/min/1.73 m2, or $\ge 3.0$ and $\le 4.5$ mmol/L if eGFR $< 45$ mL/min/1.73 m2.

**4.2 Exclusion Criteria** 1. Systolic blood pressure $> 180$ mmHg, or diastolic BP $> 110$ mmHg at screening. 2. Known hyperkalaemia, defined as potassium of $\ge 5.5$ mmol/L within 3 months at screening. 3. Serum sodium $< 135$ mmol/L (within 4 weeks prior to screening or at screening). 4. Participants with T1DM (exceptions apply for US/Japan patients treated safely with SGLT2i/dapagliflozin without DKA history). 5. Uncontrolled T2DM with HbA1c $> 10.5\%$ (within 3 months prior to screening or at screening). 6. New York Heart Association (NYHA) functional HF class IV at screening. 7. Stroke, transient ischaemic cerebral attack, valve implantation or replacement, carotid surgery or angioplasty, acute coronary syndrome, or hospitalisation for worsening heart failure within the previous 3 months. 8. Documented history of adrenal insufficiency. 9. Any dialysis or acute kidney injury (AKI) within 3 months prior to screening. 10. History of organ transplant, bone marrow transplant, or planned organ transplant within 6 months following randomisation. 11. Any clinical condition requiring systemic immunosuppression therapy other than maintenance therapy.

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## 5. Intervention & Treatment Plan

**5.1 Investigational Regimen Dosage/Frequency:** 4-week dapagliflozin Run-in Period for participants untreated with an SGLT2i at screening. Following the run-in, participants enter the double-blind period and receive either Baxdrostat + Dapagliflozin OR Placebo + Dapagliflozin. **Premature Discontinuation:** In case of premature discontinuation of blinded study intervention, participants will continue in the study and receive dapagliflozin (unless specific discontinuation criteria are met). **Route of Administration:** Oral **Duration of Treatment:** Event-driven study; treatment continues until study completion (estimated up to ~57 months).

**5.2 Concomitant & Prohibited Therapy Permitted:** Stable, maximum tolerated dose of either an ACE inhibitor or an ARB. All participants receive dapagliflozin as standard background therapy. **Prohibited:** Concomitant use of both an ACE inhibitor and an ARB simultaneously.

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## 6. Study Timeline & Visit Schedule

| Visit ID | Window (Days) | Key Activities/Procedures |
| :--- | :--- | :--- |
| **Screening / Run-in** | Day -28 to 0 | Informed Consent, Medical History, BP measurement, eGFR, baseline Potassium/Sodium Labs. Dapagliflozin run-in for 4 weeks (if applicable). |
| **Baseline** | Day 0 | Randomization, First Dose of Double-Blind IP |
| **Interim Visits** | Weeks 2, 4, 8, 16, 34, 52, then ~every 4 months | Safety Labs, Efficacy Assessment, Event tracking |
| **End of Study** | Dec 2029 (PACD) | Final Vital Signs, Final Labs, Exit Interview, IP Accountability. Scheduled within a few weeks of the Primary Analysis Cut-off Date (PACD). |

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## 7. Outcome Measures (Endpoints)

**7.1 Primary Endpoints** Time to the first occurrence of: 1. The composite endpoint of $\ge 50\%$ sustained decline in eGFR, kidney failure, or CV death. 2. The composite endpoint of kidney failure (sustained eGFR $< 15$ mL/min/1.73 m2, chronic dialysis, kidney transplant, or death with a renal primary cause). 3. CV (cardiovascular) death.

**7.2 Secondary & Exploratory Endpoints** 1. Reduction in UACR (urine albumin-creatinine ratio) after 16 weeks. 2. Reduction in SBP (systolic blood pressure) after 16 weeks. 3. Reduction in the risk of MACE (Major Adverse Cardiac Events).

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## 8. Safety & Adverse Event Reporting

**Adverse Event (AE) Monitoring:** Standard collection via regular study visits and clinical laboratory tests; specific monitoring for hyperkalemia and hyponatremia. **Serious Adverse Events (SAEs):** Standard reporting timeline, typically within 24 hours of site awareness. **Data Monitoring Committee (DMC):** Independent external adjudication and data safety monitoring committee.

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## 9. Statistical Analysis Plan (SAP) Summary

**Analysis Populations:** Intent-to-Treat (ITT) population for primary efficacy outcomes; Safety Set for safety/AE evaluations. **Sample Size Justification:** $N = 5,000$ participants, powered appropriately as an event-driven study to detect statistically significant reductions in the composite renal and CV mortality primary endpoint. **Handling of Missing Data:** Advanced imputation techniques appropriate for time-to-event and longitudinal analyses.

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## 10. Quality Assurance & Regulatory Compliance

**GCP Compliance:** This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Regular onsite and remote monitoring visits by the sponsor to assure data integrity. **Audit Trail:** eCRF locking, query resolution processes, and data clarification forms.

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## 11. Study Identifiers & Governance

**Primary ID:** D6972C00002 **Posting ID:** 870388522340004598 **Registry Number:** NCT06742723 **Sponsor/Lead Organization:** AstraZeneca **Study Title:** BaxDuo-Pacific **Official Regulatory Title:** A Phase III, Randomised, Double-blind, Placebo-controlled, Event-driven Study to Assess the Efficacy, Safety and Tolerability of Baxdrostat in Combination With Dapagliflozin Compared With Dapagliflozin Alone on Renal Outcomes and Cardiovascular Mortality in Participants With Chronic Kidney Disease and High Blood Pressure

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## 12. Clinical Framework (Study Specific)

**Primary Condition:** Chronic Kidney Disease (CKD) and Hypertension (HTN) **Preferred UMLS Name:** Hypertensive chronic kidney disease **Diagnosis Threshold:** SBP $\ge 130$ mmHg at screening; eGFR matching CKD inclusion thresholds **Medical Methodology:** Aldosterone Synthase Inhibitor (Baxdrostat) + SGLT2i (Dapagliflozin) combined therapy **Optimization Target:** Lowering blood pressure and reducing risk of renal failure and cardiovascular events

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## 13. Study Procedures & Methodology

**Management Pathway:** Double-blind, placebo-controlled event-driven clinical trial **Education Protocol (HCP):** Clinical protocol training, AE management, and IP administration guidelines **Education Protocol (Patient):** Informed consent, adherence counseling for investigational product and background medications **Quality Audit Mechanism:** Clinical trial monitoring and central event adjudication

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## 14. Observation & Follow-Up Schedule

**Total Duration:** Estimated 57 months (March 2025 to December 2029); study closure initiated at the Primary Analysis Cut-off Date (PACD) when target events are reached. **Onsite Visit Cadence:** Randomization, Weeks 2, 4, 8, 16, 34, 52, and approximately every 4 months thereafter. **Remote Monitoring Frequency:** Periodic safety check-ins and AE tracking **Checklist Review Interval:** Routine eCRF review intervals

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## 15. Sign-off & Approval

| Role | Name | Signature | Date | | :--- | :--- | :--- | :--- | | **Principal Investigator** | [Name] | ____ **| [DD-MMM-YYYY] | | Clinical Operations Lead | [Name] | ___ |** ***[DD-MMM-YYYY]*** **| |** ***Lead Statistician*** **|** ***[Name]*** **|** _____ | [DD-MMM-YYYY] |